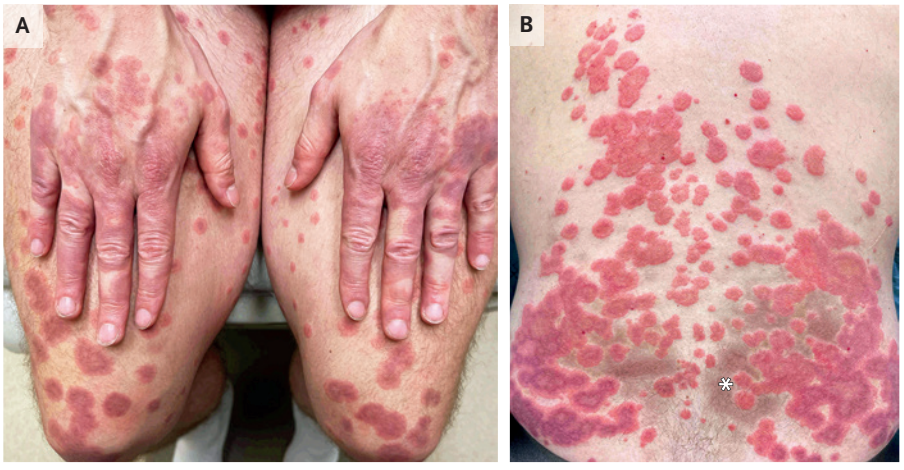
IMAGES IN CLINICAL MEDICINE

Urticarial Vasculitis



A 33-year-old man with systemic lupus erythematosus (SLE) presented to the dermatology clinic with a 1-week history of pruritic, burning plaques. Two years earlier, he had received a diagnosis of SLE, but treatment had not been started owing to mild symptoms and the absence of end-organ damage. Physical examination was notable for pink, edematous plaques on the hands, elbows, knees, and feet. Laboratory testing showed elevated antinuclear and anti–double-stranded DNA antibody titers and low complement levels. A skin-biopsy sample from the left elbow showed papillary dermal edema, intravascular neutrophils, and C3 deposition in dermal vessel walls — findings consistent with leukocytoclastic vasculitis. Treatment with oral and topical glucocorticoids was started. One week later, the lesions evolved into fixed, urticarial, annular, and targetoid plaques with rims of purpura on the thighs (Panel A), chest, abdomen, and back (Panel B), where hyperpigmentation at the site of healed lesions was also observed (asterisk). On the basis of the presence of urticarial lesions lasting more than 24 hours plus leukocytoclastic vasculitis on histopathological analysis, a diagnosis of urticarial vasculitis was made. Urticarial vasculitis may be associated with SLE (as in this patient), particularly when serum complement levels are low. Treatment with dapsone, colchicine, and hydroxychloroquine was initiated, with eventual replacement of prednisone by mycophenolate mofetil. At a follow-up visit 2.5 years after the start of treatment, the patient's disease was well controlled.

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